

DYNAMET TABLET	DESCRIPTION	CONTENT
DYNAMET TABLET 200MG MAL19940254AZ	TABLET Shape : Round, Biconvex & Scored Colour : Light Green Marking : "200" Coating : Film-coated	Each Tablet Contains: Cimetidine200 mg
DYNAMET TABLET 400MG MAL19940492AZ	TABLET Shape : Round, Biconvex & Scored Colour : Light Green Marking : "400" Coating : Film-coated	Each Tablet Contains: Cimetidine400 mg

PHARMACODYNAMICS:

Cimetidine inhibits both the stimulated and basal secretion of gastric acid and reduces pepsin output. It competitively inhibits the action of histamine at the histamine H₂-receptor of the parietal cells and thus is a histamine H₂-receptor antagonist. In addition to its antisecretory effects, Cimetidine also has cytoprotective properties and therefore has a beneficial effect in maintaining the integrity of the gastric mucosal barrier.

PHARMACOKINETICS:

Cimetidine is readily absorbed from the gastrointestinal tract and peak plasma concentrations are obtained about an hour after administration on an empty stomach and about 2 hours after administration with food.

The bioavailability of Cimetidine following oral administration is about 60% to 70% compared to an intravenous dose due to first-pass metabolism. The elimination half-life from plasma is around 2 hours and Cimetidine is weakly bound to about 20% to plasma proteins. Cimetidine is partially metabolised in the liver to the sulphoxide and to hydroxymethylcimetidine but most is excreted unchanged in the urine. Cimetidine crosses the placental barrier and is excreted into breast milk where concentrations are reported to be higher than those in plasma. It does not readily cross the blood brain barrier.

INDICATIONS:

Duodenal and gastric ulcers. Reflux oesophagitis. Pathological hypersecretory conditions such as Zollinger-Ellison syndrome. Pancreatic insufficiency.

RECOMMENDED DOSE:

The dose should be adapted to each patient according to his therapeutic response. The usual dose by mouth is 400 mg twice daily taken in the morning and at bedtime. Other regimens are 200 mg or if necessary 400 mg three times daily with 400 mg at bedtime.

For duodenal and gastric ulcer, a single dose of 800 mg is given at bedtime initially for 4 weeks in duodenal ulcer and 6 weeks in gastric ulcer. A maintenance dose of 400 mg is then given once at bedtime or both in the morning and at bedtime.

In reflux oesophagus, the recommended dose is 400 mg four times daily (with meals and at bedtime) for 4 to 8 weeks.

In pathological hypersecretory conditions, such as Zollinger-Ellison syndrome, a dose of 400 mg four times daily may also be required, occasionally increased to a total of 2.4 g daily.

For patients with pancreatic insufficiency, 0.8 to 1.6 g daily is given in 4 divided doses before meals. A suggested dose for children over 1 year of age is 25 to 30 mg per kg body weight daily in 3 or 4 divided doses.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATION:

Gastric achylia and neoplastic processes. It is best avoided in pregnant women. In cases of renal insufficiency, the dose should be reduced according to the existing renal affection.

WARNINGS AND PRECAUTIONS:

Before giving Cimetidine to patients with gastric ulcers the possibility of malignancy should be excluded since Cimetidine may mask symptoms and delay diagnosis. Cimetidine should be given in reduced dosage to patients with impaired renal function.

DRUG INTERACTIONS:

Cimetidine has been found to interact with many other drugs but the full clinical significance of some of these interactions has yet to be established. These interactions include lignocaine, phenytoin, theophylline and warfarin. Increased blood concentrations of some antiarrhythmics, some benzodiazepines, some beta-blockers and vasodilators have also been reported.

PREGNANCY AND LACTATION:

Cimetidine was detected in the milk of nursing mother in concentrations higher than in her plasma. So, Cimetidine must be carefully used in pregnant woman or during lactation period.

SIDE EFFECTS:

Adverse reactions to Cimetidine are generally infrequent and are usually reversible following a reduction of dosage or withdrawal of therapy. The commonest side-effects reported have been diarrhoea, dizziness, tiredness, and rashes. Reversible confusional states, especially in the elderly or in seriously ill patients such as those with renal failure, have occasionally occurred. Cimetidine has a weak anti-androgenic effect and gynaecomastia and impotence have also occasionally occurred in men receiving relatively high doses for conditions such as the Zollinger-Ellison syndrome. Other adverse effects which have been reported rarely are allergic reactions, arthralgia and myalgia, blood disorders including agranulocytosis or granulocytopenia and thrombocytopenia, interstitial nephritis, headache, hepatotoxicity, and pancreatitis.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptom of overdose may include slurred speech, high pulse rate, dilated pupils and bradycardia.

It is recommended that treatment of overdose of Cimetidine should consist of gastric lavage or administration of syrup of ipecacuanha, provided that not more than 4 hours have elapsed since ingestion of the drug, followed by supportive measures and symptomatic treatment only. Forced diuresis is not recommended.

PACKING/PACK SIZE(S):

DYNAMET TABLET 200MG - Blister pack of 10x10's and 50x10's.

DYNAMET TABLET 400MG - Blister pack of 10x10's and 50x10's.

JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. (65683-V)

2497, Mk. 1, Lorong Perusahaan Baru 5,

Kawasan Perusahaan Perai 3,

13600 Perai, Pulau Pinang, MALAYSIA

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